This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Orlistat 120mg capsules for the treatment of Weight management in Obesity

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Weight management in Obesity

Overview of Obesity and Weight Management

- **Definition of Obesity:** BMI ≥30 kg/m² (or ≥27.5 in some ethnic groups with increased cardiometabolic risk)
- **Associated conditions:** Type 2 diabetes mellitus, cardiovascular disease, obstructive sleep apnoea, musculoskeletal disorders

Assessment

- **Weight and anthropometric measurements:** Calculate BMI, measure waist circumference
- **Comorbidities:** Assess for associated chronic conditions and cardiovascular risk
- **Previous weight management attempts:** Document history and response to lifestyle interventions
- **Current medications:** Review for medications that may affect weight or interact with orlistat
- **Eating habits and lifestyle:** Assess diet quality, physical activity, and readiness for behaviour change

Management

- **First-line intervention:** Diet (reduced-calorie intake), physical activity (at least 150 minutes moderate-intensity per week), behaviour change
- **Pharmacological treatment:** Consider as adjunct to lifestyle changes when BMI ≥30 kg/m² OR BMI ≥28 kg/m² with associated comorbidities (T2DM, hypertension, dyslipidaemia, CVD)
- **Orlistat:** Pancreatic and gastric lipase inhibitor; reduces absorption of dietary fat

Red Flags (Refer to GP)

- **Unintentional weight loss:** May indicate underlying malignancy or other serious pathology
- **Symptoms suggestive of secondary obesity:** Cushing's syndrome, hypothyroidism, polycystic ovary syndrome, medications (corticosteroids, antipsychotics)

Patient Group Direction

for the supply/administration of

Orlistat 120mg capsules

for the treatment of

Weight management in Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing

	organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to reduced-calorie diet and lifestyle changes in the management of obesity in adults with BMI ≥30 kg/m² or BMI ≥28 kg/m² with associated comorbidities such as Type 2 diabetes mellitus, hypertension, dyslipidaemia or cardiovascular disease.
Inclusion criteria	Age ≥18 years and <75 years BMI ≥30 kg/m² OR BMI ≥28 kg/m² with at least one obesity-related comorbidity (T2DM, hypertension, dyslipidaemia, CVD) Motivated and committed to weight loss with structured diet (reduced-calorie diet, typically 500-1000 kcal below estimated daily expenditure) Informed consent obtained; patient understands medication mechanism, expected outcomes, and common side effects Baseline weight and waist circumference documented
Exclusion criteria	Chronic malabsorption syndrome (e.g., cystic fibrosis, coeliac disease, inflammatory bowel disease) Cholestasis or severe hepatic impairment Pregnancy or breast-feeding Concurrent ciclosporin therapy (reduced absorption of ciclosporin) Warfarin therapy (relative contraindication; requires specialist assessment) Concurrent antiepileptic medicines, antiretroviral medicines for HIV, or amiodarone, or any other medicine with an SmPC interaction that cannot be managed in a pharmacy setting (orlistat may reduce their absorption or plasma levels and unbalance treatment); refer to the GP or specialist Known hypersensitivity to orlistat or any component of the

	formulation Age <18 years or ≥75 years Uncontrolled or newly diagnosed diabetes (require GP review before starting)
Cautions	Diabetes mellitus: Blood glucose control may improve; dose of antidiabetic medication (especially insulin and sulfonylureas) may require adjustment Anticoagulant therapy other than warfarin (edoxaban, dabigatran, rivaroxaban, apixaban): Enhanced anticoagulant effect possible; requires GP liaison. Warfarin is an exclusion (see exclusion criteria) Fat-soluble vitamin absorption: May be reduced (vitamins A, D, E, K); recommend multivitamin supplementation at least 2 hours apart from orlistat Hypothyroidism: Levothyroxine absorption may be reduced; monitor thyroid function and dose; administer levothyroxine at least 4 hours before orlistat History of oxalate kidney stones: Risk of hyperoxaluria and recurrence Chronic liver disease or elevated liver function tests: Proceed with caution; ensure baseline LFTs checked Gallstone disease or bile acid sequestrants: Monitor for interactions and symptoms
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Orlistat 120mg capsules
Legal category	POM (Prescription-Only Medicine)
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral administration; swallow capsule whole with a glass of water with or shortly before each main meal.
Dose and frequency	120mg with each main meal containing fat (up to 3 times daily, typically with breakfast, lunch, and dinner) If a meal is missed or contains negligible fat, omit the dose Maximum: 360mg daily (3 × 120mg)

Quantity to be administered and/or supplied	Up to 84 capsules per patient (28-day supply at maximum dose of 3 capsules daily)
Maximum or minimum treatment period	Review at 12 weeks (3 months) from start of treatment. Continue treatment only if patient has achieved ≥5% reduction in body weight from baseline. If weight loss target not achieved, discontinue and refer to GP for alternative management strategies.
Adverse effects	Common (≥1% in clinical trials): Oily spotting, flatulence with discharge (faecal incontinence), faecal urgency, fatty or oily stool, increased defaecation, abdominal pain, headache Uncommon: Diarrhoea, upper respiratory tract infection, lower respiratory tract infection, anxiety, fatigue, nausea Rare: Hepatitis, pancreatitis, oxalate nephropathy (hyperoxaluria), cholesterol gallstone formation, rash, pruritus The incidence and severity of GI adverse effects typically decrease over time as patients adjust their fat intake

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form, route and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the Patient Information Leaflet (PIL) provided with Orlistat. Ensure patient understands mechanism of action, dietary requirements, and expected side effects.

Follow-up advice to be given to patient or carer	Maintain a reduced-calorie diet with low fat content (≤30% of daily calories from fat) to minimise GI side effects such as oily spotting, flatulence, and stool urgency. Take a multivitamin supplement containing fat-soluble vitamins (A, D, E, K) at least 2 hours apart from orlistat to ensure adequate vitamin absorption. Do not take orlistat if a meal is missed or contains no fat (e.g., fat-free meal). If taking levothyroxine, administer at least 4 hours before orlistat to avoid reduced absorption. Report any jaundice, persistent abdominal pain, signs of pancreatitis, or persistent diarrhoea to the GP immediately. Expect weight loss to be gradual (typically 2-4 kg in the first 12 weeks if combined with a reduced-calorie diet and exercise). If diabetes medications are being taken, inform the GP as blood glucose control may improve and medication adjustment may be needed. Attend follow-up appointment at 12 weeks to assess weight loss progress (≥5% target); continue treatment only if this target is met. If on anticoagulant therapy, ensure the GP is aware. Report any suspected adverse drug reactions or side effects to the GP or via the Yellow Card scheme at https://yellowcard.mhra.gov.uk.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Weight management and obesity
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			

Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

GET REAL HEALTH

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 54: an exclusion is added for concurrent antiepileptic medicines, antiretroviral medicines for HIV, amiodarone, or any other medicine with an SmPC interaction that cannot be managed in a pharmacy setting, matching the consultation tool.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from administration to supply; the capsules are supplied.

Cautions and follow-up advice: warfarin removed from the anticoagulant caution and the patient advice because warfarin therapy is an exclusion; the caution now covers the direct oral anticoagulants only.

Adverse effects: the duplicated faecal urgency entry was removed.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

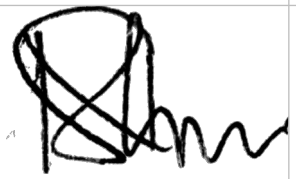
Changes in this version:

First reissue of this document.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.